

Benchmark - Counselor Ethical Boundaries and Practices

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Professional counseling carries ethical and legal obligations that shape every interaction with clients, colleagues, and supervisors. The ACA Code of Ethics (2014) provides the foundation, but applying its standards depends on the counselor's judgment, self-awareness, and willingness to consult. This paper examines five interrelated areas of ethical practice: the legal and ethical considerations of telehealth and digital service delivery, boundary issues and dual relationships, professional collaboration within multidisciplinary teams, the supervisor-counselor relationship and counselor response to unethical behavior, and the role of self-care, self-awareness, and self-evaluation in sustaining ethical practice across a counselor's career.

Ethical and Legal Considerations

The expansion of telehealth and digital service delivery has introduced ethical and legal complexities absent from face-to-face counseling. Section H of the ACA Code addresses these concerns directly, recognizing that distance counseling and technology carry unique risks (American Counseling Association [ACA], 2014). Counselors using video platforms, client portals, or messaging applications hold the same ethical obligations as in-person counselors, with added responsibilities tied to the technology.

Confidentiality in Telehealth

Virtual confidentiality depends on technology that can fail or be intercepted. Counselors must take reasonable precautions to protect electronic information (ACA, 2014, Standard B.3.e), which means using HIPAA-compliant video platforms, encrypting records, and assessing the client's environment for privacy. A common breach occurs when a client conducts a session from a household where others can overhear, or when the counselor holds a session in a shared office without sound-masking measures in place.

Informed Consent for Digital Modalities

Telehealth informed consent must include disclosures beyond face-to-face practice. Clients need to understand the platform, its limitations, the protocol for interrupted sessions, the counselor's jurisdiction, and how emergencies will be handled (ACA, 2014, Standards H.2.a, H.2.c). This typically takes the form of a telehealth-specific consent reviewed during the first session and revisited when modalities change.

Record Keeping

Digital service delivery generates records across video files, chat logs, EHR entries, and messages. Counselors must ensure these records remain secure and accessible only to authorized persons (ACA, 2014, Standard B.6.b), using encrypted storage, multi-factor authentication, and state-required retention schedules. Practices such as screenshotting client messages onto a personal phone create serious legal exposure.

Jurisdictional Issues

Telehealth allows cross-border practice, but licensure is granted at the state level. DeDiego et al. (2023) emphasize that practicing across state lines without authorization can constitute unauthorized practice and create risk for both client and counselor. The Counseling Compact addresses this barrier among participating states. Until adoption is widespread, counselors must verify the client's location at each session and confirm jurisdictional authority. A client who relocates mid-treatment can place a counselor out of compliance overnight.

Boundary Issues and Dual Relationships

Few areas of counseling are as nuanced as managing boundaries and dual relationships. The ACA Code is clear on absolute prohibitions such as sexual relationships with current clients (American Counseling Association [ACA], 2014, Standard A.5.a), but most boundary questions

demand judgment, consultation, and documentation rather than rule-following. Corey et al. (2024) note that no consensus exists regarding nonsexual multiple relationships.

Criteria for Decision Making

When evaluating a potential nonprofessional relationship, the counselor's first questions concern necessity, harm, and benefit. Drawing on Younggren and Gottlieb's framework in Corey et al. (2024), the counselor should ask whether the relationship is avoidable, whether it could harm the client, who benefits, whether it could disrupt the alliance, and whether objectivity can be maintained. Barnett and colleagues add consideration of culture, history, diagnosis, and treatment plan (as cited in Corey et al., 2024). Standard A.6.e requires documentation of the rationale (ACA, 2014). If the counselor cannot defend the decision to a supervisor, that absence of defensibility is itself the answer.

Crossing Versus Violation

A boundary crossing is a departure from common practice that may benefit the client, such as a supportive handshake or appropriate self-disclosure. A boundary violation is a serious breach that harms the client, such as sexual involvement or financial exploitation (Gutheil & Gabbard, 1993, as cited in Corey et al., 2024). As Barnett (2017) observed, "one client's boundary crossing may be another client's boundary violation" (as cited in Corey et al., 2024, p. 301). Ongoing consultation and documentation prevent crossings from drifting into violations.

Applied Examples

Physical attraction between counselor and client. Attraction may be mutual, one-sided from the counselor, or expressed by the client. Mutual attraction triggers Standard A.5.a and, under Standard C.2.g, may constitute professional impairment requiring consultation and possible referral (ACA, 2014). One-sided counselor attraction is not unethical but requires

supervision and, when needed, personal therapy (Corey et al., 2024). Client-expressed attraction is treated as clinical material, explored without shame and documented carefully.

Recovery meeting overlap. In small or rural communities, a counselor in recovery may encounter a client at the same recovery meeting (Zur, 2007). Avoidance is often impossible; the counselor must discuss the situation openly with the client, clarify that the meeting is not a counseling space, and document the conversation.

Post-termination friend request. A former client sending a social media request raises questions about timing and harm. Gottlieb's (1993) decision-making model weighs the power differential, duration of the prior relationship, and likelihood of renewed contact. In most cases, the safer answer is to decline.

Bartering for services. Standard A.10.e permits bartering only when it is non-exploitative, client-requested, community-accepted, and documented in writing (ACA, 2014). Even when these conditions are met, the counselor must remain vigilant about whether the exchange disrupts the therapeutic dynamic.

Professional Collaboration in Counseling

Effective counseling in agency settings rarely occurs in isolation. As a future counselor entering practice as an LPC-A in a community mental health agency, I anticipate working alongside a multidisciplinary team to ensure coordinated care. Section D of the ACA Code emphasizes that counselors should develop relationships with colleagues across disciplines and maintain focus on serving the client (American Counseling Association [ACA], 2014, Standards D.1.b, D.1.c).

Working Within Scope of Practice

Collaboration begins with clarity about one's own scope. As a counselor, my role is to provide psychotherapy and mental health support, not to prescribe medication, deliver medical care, or perform formal psychological testing. The ACA Code requires counselors to respect the contributions of other disciplines while maintaining their own ethical obligations (ACA, 2014, Standard D.1.e). Recognizing my limits allows me to refer appropriately and avoid overstepping.

Multidisciplinary Team Roles

A community mental health team typically includes a psychiatrist or psychiatric nurse practitioner managing medication and assessing medical contributors to symptoms; a case manager helping clients access housing, transportation, and benefits; a social worker handling discharge planning and family interventions; and primary care providers, when integrated, supporting medical screening. My role as the counselor is to provide therapeutic services, contribute to treatment planning, and communicate relevant clinical information to the team while protecting client confidentiality. Effective collaboration depends on regular communication, mutual respect, and keeping the client at the center of every decision.

Relationships with Supervisors and Colleagues

Clinical supervision is one of the most influential relationships in a counselor's professional development. The ACA Code dedicates Section F to supervision, training, and teaching, recognizing that supervision quality directly affects both client welfare and supervisee growth (American Counseling Association [ACA], 2014).

Role and Responsibilities of the Clinical Supervisor

The clinical supervisor's primary responsibilities are to protect client welfare, support supervisee development, serve as a gatekeeper for the profession, and prepare the supervisee for

independent practice (Corey et al., 2024, p. 383). To do this, supervisors monitor client welfare and clinical performance, meet regularly with supervisees, and provide ongoing feedback (ACA, 2014, Standard F.1.a). Supervisors operate as teacher, mentor, consultant, evaluator, and documenter, and they hold ethical and legal accountability for their trainees' work (Corey et al., 2024, p. 388).

Ethical Issues in the Supervisor-Counselor Relationship

The supervisory relationship contains an inherent power differential. Supervisors must remain aware of this dynamic and avoid exploiting it while fulfilling their evaluative and gatekeeping responsibilities. A written supervision contract, clear documentation, and informed consent help structure the relationship (Corey et al., 2024, p. 394). Supervisors also carry vicarious liability for their supervisees' actions, making case review essential (Corey et al., 2024, p. 395).

Comparison to the Counselor-Client Relationship

The supervisor-counselor relationship shares features with counseling: trust, working alliance, protected communication, and a power differential the more powerful party must manage. The key differences are that supervision is explicitly evaluative and gatekeeping, and the supervisor is accountable to a third party, the client, whose welfare depends on the supervisee's competence.

Responding to Unethical Behavior in a Colleague

Counselors sometimes observe unethical behavior such as confidentiality breaches, practicing outside competence, or impairment. The ACA Code directs counselors to attempt informal resolution when substantial harm has not occurred and confidentiality permits (ACA, 2014, Standard I.2.a). When the situation is serious, consultation with a knowledgeable colleague

or ethics authority is required (ACA, 2014, Standard I.2.c). Corey's eight-step ethical decision-making model provides a framework: identify the problem, identify potential issues, review ethical guidelines, consider applicable laws, obtain consultation, weigh courses of action, enumerate consequences, and decide on the best course (Corey et al., 2024). Walking through these steps slows the counselor down, forces consultation, and creates a written record of how the decision was made.

Self-Awareness and Self-Evaluation

Becoming an ethical counselor depends not only on training but on the ongoing internal work of self-care, self-awareness, and self-evaluation. The ACA Code addresses this in Standard C.2.g, which requires counselors to monitor themselves for signs of impairment and refrain from offering services when impaired (American Counseling Association [ACA], 2014).

The Value of Self-Care and Self-Awareness

Self-care is often framed as optional, but in counseling it is an ethical imperative. A counselor who is depleted or unaware of their own emotional state cannot offer clients the steady, attuned presence the work requires. For me, this means prioritizing personal therapy, sleep, exercise, and physical health. Bevly and Prosek (2024) found that counselors who engage in their own therapy report deeper self-understanding and greater capacity for empathic presence. Self-awareness builds on those foundations and allows me to recognize when a client's material is touching something in me or when I am operating on fumes. Corey et al. (2024) emphasize that self-awareness is a continuous practice, not a destination.

Self-Evaluation in Ethical Practice

Self-evaluation is the bridge between self-awareness and ethical action. Supervision and consultation are my most important self-evaluation tools; bringing cases to a supervisor or

trusted peer offers a perspective I cannot generate alone and helps me catch blind spots before they affect a client. Client feedback is another tool I plan to use intentionally because clients are the experts on their own lives. Choi and Hyun (2023) note that self-evaluation grounded in mindfulness helps counselors stay connected to their own values and to the needs of their clients. Without ongoing self-evaluation, even a well-trained counselor can drift into habits that no longer serve clients.

Conclusion

Across the five areas examined in this paper, a common theme emerges. Ethical counseling is not a checklist but a practice that requires continuous engagement with the ACA Code, with colleagues, and with the counselor's own internal life. The technological tools that shape service delivery, the relational complexity of boundary decisions, the realities of working within a multidisciplinary team, the power dynamics of supervision, and the inner work of self-care and self-evaluation all point to the same principle. Without consultation, documentation, and honest reflection, ethical practice does not hold up under pressure. As I move toward licensure, I am committed to carrying those practices into clinical work.

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